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OFFICIAL NDIS REPORT

NDIS Psychosocial Assessment - Johnathan Miller

Report Date: 2026-06-20
Participant ID: 6XGFASFF

NDIS PARTICIPANT DETAILS

PARTICIPANT NAME
Johnathan Miller

NDIS PARTICIPANT ID
N123456789

PARTICIPANT EMAIL
j.miller@example.com.au

DATE OF BIRTH (DOB)
2000-09-11

Official NDIS Audit & Progress Findings

PSYCHOSOCIAL ASSESSMENT REPORT

Practice Details:

Aleksandra Gorczynska-Kolodziej
ABN: 33257573623
ACA Registered Counsellor (Level II)
Email: office@horizonscc.com.au | Phone: 0401413634
Report Date: 2026-06-20

Here is the updated information block for Johnathan Miller. I have ensured all details match the client context data you provided! =

PARTICIPANT INFORMATION

FIELD	DETAILS
Full Name	Johnathan Miller
NDIS Number	N123456789
Date of Birth	11/09/2000
Support Coordinator	Sarah Mitchell (sarah.mitchell@futuresupport.com.au / 0412 345 678)
Plan Management	Plan Managed
Assessment Period	20/06/2026

1. EXECUTIVE SUMMARY

Purpose of Assessment:

Comprehensive psychosocial assessment to evaluate participant's mental health, emotional wellbeing, social connectedness, and coping capacity. This assessment informs NDIS planning for psychosocial and therapeutic supports under Capacity Building - Improved Daily Living.

Assessment Methods:

- 2x Clinical Interviews (4 hours total)
- Standardised psychosocial screening tools (AQ-10, GAD-7, PHQ-9, DASS-21, K10, WHODAS 2.0, LSNS-6, Brief-COPE, WEMWBS, ISI)
- Mental State Examination
- Collateral consultation with support network
- Narrative therapeutic consultation

Key Psychosocial Findings:

Participant presents with significant psychosocial distress characterised by severe anxiety, moderate depression, and limited coping strategies. Participant demonstrates characteristics consistent with neurodivergent presentation (AQ-10 score indicates high likelihood of ASD), including difficulties with social communication, sensory sensitivities, and rigid behavioural patterns. These factors substantially impact participant's capacity to engage in social, community, and daily living activities without support.

Total Recommended Psychosocial Support & NDIS Line Items:

- **Therapeutic Counselling:** 1 hour/week for 12 weeks

(NDIS Line Item: Assessment Recommendation Therapy or Training - Counsellor, Item 15_043_0128_1_3)

- **Psychosocial Recovery Coach / Support Worker:** 3 hours/week

(NDIS Line Item: Psychosocial Recovery Coach, Item 04_105_0125_6_1 OR Assistance with Social, Economic and Community Participation)

- **Review Period:** 6 months

2. PSYCHOSOCIAL ASSESSMENT TOOLS ADMINISTERED

TOOL	DOMAIN	SCORE	INTERPRETATION
AQ-10	Autism Screening	22/50	High likelihood of neurodivergent presentation - formal diagnostic assessment recommended
GAD-7	Anxiety	18/21	Severe Anxiety
PHQ-9	Depression	14/27	Moderately Severe Depression
DASS-21	Depression / Anxiety / Stress	D:14 A:18 S:20	Severe Anxiety, Moderately Severe Depression, Severe Stress
K10	Psychological Distress	38/50	Very High Psychological Distress
WHODAS 2.0	Functional Impact	48/60	Severe Functional Impairment
LSNS-6	Social Network	4/18	Severely Limited Social Network
Brief-COPE	Coping Strategies	Predominantly Avoidant	Maladaptive Coping Profile
WEMWBS	Mental Wellbeing	28/70	Low Mental Wellbeing
MSPSS	Perceived Support	18/49	Low Perceived Support
C-SSRS	Suicide Risk	Category 2	Moderate Risk - Ideation without Intent
ISI	Sleep	16/28	Moderately Severe Insomnia

3. PSYCHOSOCIAL FUNCTIONING ASSESSMENT

DOMAIN 1: MENTAL HEALTH & EMOTIONAL WELLBEING

STATUS: SEVERE IMPAIRMENT

Clinical Observations:

- Participant reports chronic anxiety related to social situations, sensory sensitivities, and disruption to routines
- GAD-7 score of 18 indicates Severe Anxiety symptoms
- PHQ-9 score of 14 indicates Moderately Severe Depressive symptoms
- Mental State Examination reveals: Anxious affect, concrete thinking style, limited eye contact, detailed and precise speech patterns, evidence of repetitive self-soothing behaviours when anxious
- WEMWBS score of 28 indicates Low Mental Wellbeing
- ISI score of 16 indicates Moderately Severe Insomnia affecting daily functioning

Psychosocial Impact Statement:

Due to severe anxiety and depressive symptoms, participant experiences significant emotional distress that impacts their capacity to engage in social interactions, maintain employment, and manage daily routines independently. Sensory sensitivities compound anxiety in public spaces and social settings.

Recommendations:

- Therapeutic counselling focusing on Cognitive Behavioral Therapy adapted for neurodivergent presentation, trauma-informed, or narrative approaches
- Psychoeducation on anxiety management and emotional regulation
- Regular monitoring of psychological distress levels

DOMAIN 2: SOCIAL CONNECTION & RELATIONSHIPS

STATUS: SEVERE ISOLATION

Clinical Observations:

- LSNS-6 score of 4 indicates Severely Limited Social Network
- MSPSS score of 18 indicates Low Perceived Social Support
- Participant reports: "I don't understand what people expect from me in conversations. I feel like I'm always doing something wrong."
- WHODAS interpersonal relationships domain: Severe difficulty (score 4/5)
- Participant has minimal social contact outside of family. Reports no close friends. Avoids social gatherings due to sensory sensitivities and uncertainty about social expectations.

Psychosocial Impact Statement:

Participant experiences social isolation that compounds mental health difficulties. Limited social support reduces resilience and increases vulnerability during periods of distress.

Recommendations:

- Psychosocial support worker / Recovery Coach to facilitate community connection
- Graduated social engagement with therapeutic support
- Goal: Develop at least 2 meaningful social connections within 6 months

DOMAIN 3: COPING & RESILIENCE

STATUS: SIGNIFICANT DEFICITS

Clinical Observations:

- Brief-COPE profile indicates Predominantly Avoidant coping strategies
- Participant reports: "When things get overwhelming, I just shut down and hide in my room. I don't know what else to do."
- Current coping strategies: Social withdrawal, repetitive behaviours, avoidance of triggering situations
- Limited repertoire of adaptive coping strategies

Psychosocial Impact Statement:

Participant has limited repertoire of adaptive coping strategies, relying primarily on avoidance and withdrawal. This limits capacity to manage stress and emotional challenges independently.

Recommendations:

- Skills-based counselling to develop adaptive coping strategies
- Psychoeducation on emotion regulation techniques
- Practice opportunities with psychosocial support worker

DOMAIN 4: DAILY PSYCHOSOCIAL FUNCTIONING

STATUS: SEVERE IMPAIRMENT

Clinical Observations:

- WHODAS overall score of 48 indicates Severe Functional Impairment
- Participant requires support with community access, social participation, employment tasks, household management
- Impact on: self-care, household management, community access, social participation
- Sleep disruption (ISI score: 16) affecting daily functioning
- Participant currently not employed due to anxiety and sensory sensitivities in workplace
- Rigid behavioural patterns create distress when routines are disrupted

Psychosocial Impact Statement:

Psychosocial difficulties substantially impact participant's capacity to engage in daily activities. Without support, participant experiences social isolation, unemployment, and inability to maintain consistent daily routines.

Recommendations:

- Psychosocial support worker assistance with community access, skill practice, and routine building
- Therapeutic work on behavioural activation and flexible thinking
- Sleep hygiene strategies integrated into counselling

DOMAIN 5: TRAUMA & SAFETY (if applicable)

STATUS: MODERATE CONCERN

Clinical Observations:

- Participant reports history of bullying and social rejection related to neurodivergent presentation
- Current safety: No immediate risk of harm to self or others
- Participant reports feeling "different" and "wrong" throughout life, contributing to low self-esteem and social anxiety
- No history of formal trauma, but chronic social stress has created trauma-like responses in social situations

Psychosocial Impact Statement:

History of social rejection and bullying contributes to current psychosocial difficulties. Trauma-informed approach required in all support interactions.

Recommendations:

- Trauma-informed counselling (phase-oriented approach)
- Safety planning and stabilisation before addressing social anxiety
- All providers to be informed of trauma-informed care principles

4. MENTAL HEALTH RISK ASSESSMENT

RISK DOMAIN	LEVEL	DETAILS
Suicide/Self-Harm	MODERATE	C-SSRS: Category 2 - Ideation without intent. Participant reports passive thoughts of "wanting to disappear" during periods of overwhelming anxiety, but no active suicidal ideation or self-harm behaviours. Protective factors: strong attachment to mother, special interests provide meaning. Safety plan: Prepared.
Self-Neglect	MODERATE	Requires monitoring of inconsistent self-care routines, particularly during periods of low mood.
Vulnerability	HIGH	Social isolation increases vulnerability. Participant reports difficulty recognising when others may be taking advantage of them.
Hospitalisation Risk	LOW	No current indicators of acute mental health crisis requiring hospitalisation.

Safety Plan: Prepared and reviewed with participant

Crisis Contacts Provided: Lifeline (13 11 14), Suicide Call Back Service (1300 659 467)

GP Informed: Yes - participant's GP has been notified of assessment findings and recommendations

5. THERAPEUTIC RECOMMENDATIONS

SHORT-TERM GOALS (0-3 months)

GOAL #	NDIS GOAL ALIGNMENT	THERAPEUTIC INTERVENTION	FREQUENCY	DURATION	SUCCESS INDICATOR
1	"I want to manage my anxiety better"	CBT-based anxiety management adapted for neurodivergent presentation	Weekly	12 sessions	GAD-7 reduction from 18 to <12
2	"I want to understand myself better"	Psychoeducation on neurodivergent presentation and strengths-based approach	Integrated	Ongoing	Participant demonstrates increased self-understanding
3	"I want to sleep better"	Sleep hygiene intervention	Integrated	8 sessions	ISI reduction from 16 to <10

LONG-TERM GOALS (3-12 months)

GOAL #	TARGET OUTCOME	TIMELINE
4	Sustainable mental wellbeing (WEMWBS >40)	6-9 months
5	Development of meaningful social connections	6-12 months
6	Independent use of adaptive coping strategies	12 months

6. PSYCHOSOCIAL SUPPORT STRATEGIES FOR SUPPORT WORKERS

TOTAL RECOMMENDED HOURS: 3 hours/week

Breakdown:

ACTIVITY	HOURS/WEEK	DESCRIPTION
Community Connection	1 hr	

		Facilitating social engagement in low-sensory environments, accompanying to community activities aligned with special interests
Emotional Support	0.5 hrs	Active listening, compassionate presence, encouragement during difficult moments
Skill Practice	1 hr	Practicing coping strategies in real-world settings
Daily Living Support	0.5 hrs	Prompting and support with routines
TOTAL	3 hrs/week	

PSYCHOSOCIAL SUPPORT APPROACH

DO:

- Use person-centred, trauma-informed approach
- Practice active listening and emotional validation
- Encourage participant's autonomy and decision-making
- Support gradual exposure to anxiety-provoking situations (at participant's pace)
- Reinforce use of coping strategies learned in counselling
- Maintain consistent, predictable interactions (builds trust)
- Report changes in mood or wellbeing to counsellor
- Respect participant's pace and boundaries
- Provide clear, explicit communication (avoid sarcasm, idioms)
- Offer advance notice of changes to routines or plans

DON'T:

- Rush participant into overwhelming situations
- Dismiss or minimise emotional experiences
- Make decisions for participant (promote dependence)
- Ignore signs of emotional distress
- Create unpredictable or inconsistent routines
- Push beyond participant's window of tolerance
- Use sarcasm, idioms, or implicit communication without explanation
- Force eye contact or physical contact

EMOTIONAL SUPPORT STRATEGIES

When participant is anxious:

1. Provide calm, reassuring presence
2. Use grounding techniques (5-4-3-2-1 sensory awareness)
3. Encourage slow breathing (4-7-8 technique)
4. Offer choice and control over situation
5. Validate emotional experience

When participant is low/depressed:

1. Gently encourage activity (behavioural activation)
2. Break tasks into small, manageable steps
3. Celebrate small achievements
4. Maintain warm, non-judgmental presence
5. Monitor for worsening symptoms (report to counsellor)

When participant is withdrawn:

1. Respect need for space while maintaining connection
2. Use gentle, non-intrusive check-ins

3. Offer low-demand activities (e.g., walk, quiet company)
4. Avoid pressure to "cheer up"
5. Report prolonged withdrawal to counsellor

7. COORDINATION REQUIREMENTS

Actions Required from Support Coordinator:

[] IMMEDIATE (Within 1 week):

- Implement recommended psychosocial support worker hours (3 hrs/week) under Assistance with Social, Economic and Community Participation OR Psychosocial Recovery Coach (Item 04_105_0125_6_1)
- Source counsellor for therapeutic sessions (12-session block) under Assessment Recommendation Therapy or Training - Counsellor (Item 15_043_0128_1_3)
- Review NDIS plan for adequate Capacity Building funding (Improved Daily Living)
- Provide referral to registered psychologist for formal diagnostic assessment if appropriate
- Provide referral to Occupational Therapist for sensory profile assessment if appropriate

[] WITHIN 1 MONTH:

- Coordinate initial meeting between counsellor and support workers
- Ensure all providers understand psychosocial support approach
- Confirm service agreements signed

[] ONGOING (Monthly):

- Monitor participant engagement with services and mental health stability
- Quarterly review of progress with counsellor

[] BEFORE PLAN REVIEW (3 months prior):

- Request updated psychosocial progress report from counsellor
- Compile evidence for NDIA plan review and submit review request

8. REVIEW TIMELINE

REVIEW TYPE	TIMELINE	ASSESSMENT TOOLS	SUCCESS INDICATORS
Clinical Review	3 months	GAD-7, PHQ-9, WEMWBS	Symptom reduction, improved wellbeing
Progress Report	6 months	Full reassessment	Functional improvements
Plan Review	12 months	Comprehensive review	Evidence for NDIA

Report Validity: This assessment is valid for 12 months for NDIS planning purposes.

9. SCOPE OF PRACTICE STATEMENT CRITICAL

This assessment was conducted by a registered counsellor within their professional scope of practice. It focuses strictly on psychosocial and mental health functioning.

This report DOES NOT constitute:

- LA Functional Behavioural Assessment (FBA)
- LA Behaviour Support Plan (BSP)
- LA clinical psychological diagnosis (e.g., specific DSM-5/ICD-11 diagnosis)
- LA neuropsychological assessment

- LA psychiatric assessment
- LA occupational therapy sensory profile assessment
- LA assessment of physical functioning, mobility needs, or home modification requirements. *(To be assessed by an Occupational Therapist if required).*

For the following, separate referral is recommended:

- Behavioural support needs ' Positive Behaviour Support Practitioner (PBS)
- Formal psychological diagnosis ' Registered Psychologist (AHPRA)
- Formal sensory profile assessment ' Occupational Therapist
- Psychiatric assessment/medication ' Psychiatrist
- Occupational therapy needs ' Occupational Therapist
- Speech pathology needs ' Speech Pathologist

Collaborative Practice:

This counsellor is available to collaborate with PBS practitioners, psychologists, OTs, and other allied health professionals to provide integrated support for the participant.

10. CLINICIAN DETAILS

- **Name:** Aleksandra Gorczynska-Kolodziej
- **Professional Registration:** ACA Member (Registration No: 86625)
- **NDIS Worker Screening Check:** [x] Cleared
- **Professional Indemnity Insurance:** [x] Current
- **Blue Card:** [x] Current
- **Professional Description:** Registered clinical counsellor specializing in cognitive behavioral therapy (CBT), psychodynamic approach, mental health care, family and system therapies, and comprehensive psychosocial assessments.

Contact:

- **Email:** office@horizonscc.com.au
- **Phone:** 0401413634

Signature: Aleksandra Gorczynska-Kolodziej

Date: 2026-06-20